



A Population Out of Control? Myths About Muslim Fertility in Contemporary India

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Summary. — This paper describes and criticizes myths about the scale and causes of fertility differentials between Hindus and Muslims in India. These ideas, associated with Hindu nationalist organizations, also have a more general common-sense quality. The paper challenges these views by examining how demographers have addressed Hindu–Muslim fertility differences, considering the impact of regional differences, variations in socioeconomic position, and occupation. We further suggest that these elaborate statistical analyses on large-scale data sets are not readily sensitive to local-level variations. Our micro-level research findings are used to illuminate both Hindu Right political rhetoric and the limitations of macro-level demographic analyses.

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1. INTRODUCTION

*Charan Singh:*¹ I have three sons—the third came in the foolishness of looking for a girl. I will try to educate them all to MA level in the hopes they will get service. It does not matter if they all go away to work, I can always employ someone to do the farm work. Unless they get a good job, what benefit will there be from the education? Fortunately, my wife can supervise the children's study; I myself do not have the time.

Roger: Why did you want a girl?

Charan Singh: First to help her mother in the house, before she is married; secondly because if there is any work to be done (like getting a glass of water or some food) a girl will never refuse but a son will; also a daughter is needed for me to get the merit of giving a daughter in marriage.

Roger: Why not have only one son, then he would get all the land?

Charan Singh: Like I said, I need more than one in case that one son is bad.

Roger: Then why not have many more sons?

Charan Singh: Yes, that would be good for making the country strong; and would be important, for example in fighting, like against the Muslims, because their population is growing faster. But children are too expensive: the everyday costs are so high I could not afford any more.

Fertility in India has been declining since the early 1970s and numerous recent studies indicate that many people say that they can afford only a few children. In the same period, family planning has been ineluctably flavored by the communalization of politics at both national and grassroots levels, with the widespread belief that Indian Muslims oppose family planning and do not adhere to the often-expressed ideal of “two boys and a girl.” Thus, this interchange with a Hindu middle-to-rich peasant

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farmer from the locally dominant Jat caste in Bijnor district (Uttar Pradesh, north India) encapsulates important themes in current understandings of India's fertility transition—a transition that seems to be slower in UP than in most of the rest of India.

This paper outlines “myths of population out of control,” especially those about the scale and causes of fertility differentials between Hindus and Muslims that are associated with the so-called Hindu Right. These ideas have a common-sense quality, a pervasive but misplaced credibility, even for people with no active involvement in Hindu Right politics. The second section begins to challenge these views by examining how demographers have addressed Hindu–Muslim fertility through themes like regional differences, variations in socioeconomic position, and occupation rather than religion as such. We are broadly sympathetic to their approach, yet demographers' elaborate statistical analyses on large-scale data sets are not readily sensitive to local-level variations. To conclude, we turn to our research in rural Bijnor to shed light both on Hindu Right political rhetoric and on the limitations of macro-level demographic analyses.

2. COMMUNAL POLITICS AND THE NUMBERS GAME

The Hindu Right comprises several organizations, including the Bharatiya Janata Party (BJP), Rashtriya Swayamsevak Sangh (RSS or National Volunteer Corps), Vishwa Hindu Parishad (VHP) and their associated women's and youth organizations.² These organizations epitomize an explicitly Hindu nationalist stance (as distinct from Nehru's “secular” nationalism). For instance, during the 1980s, they spearheaded a campaign to remove the Babri Masjid (a 16th century mosque in Ayodhya), claiming that it was built over the birthplace of the Hindu god Ram. Their supporters demolished the mosque in 1992 and anti-Muslim riots flared around India.³ The BJP (the Hindu Right's main political party) and allied parties was in power in UP from 1997–2002, and its leader has been the Indian Prime Minister since 1998.

During the 1980s and 1990s, the Hindu Right echoed allegations repeatedly made in Hindu nationalist discourse since the early 20th century that Muslims constitute a threat to the “Hindu

community” (Datta, 1993). A heightened preoccupation with “community” numbers and population growth rates has been linked to colonial census operations and other aspects of British rule, especially from the latter part of the 19th century onwards (Appadurai, 1993; Cohn, 1987; Jones, 1981; Pandey, 1990). During the 20th century, Hindu nationalist claims have focused inter alia on Muslim expansionism through invasions, forced conversions, and the abduction of Hindu women and have characterized Muslim men as militant, sexually predatory, and a danger to India's integrity (Bacchetta, 1994, 1996; Basu, 1993; Gupta, 1998, 2002, 222–320; Jaffrelot, 1995; Pandey, 1991). This discourse also portrays Indian Muslims as complicit with Pakistan, with their efforts to boost Muslim numbers supposedly entailing the conversion of poor Hindus (financed with Gulf state money) and schemes to infiltrate Muslims from Pakistan and Bangladesh into India (Basu, 1996, pp. 132–135; Basu *et al.*, 1993, pp. 74–75; Gupta & Sharma, 1996; Jaffrelot, 1996, pp. 338; Wright, 1983).⁴

To argue on the Hindu Right's chosen ground of contrasting the demographic profiles of religious communities risks conceding that “the philosophical postulates of a particular religion... constitute the exclusive, unchanging organizational principles for an entire people across all kinds of spaces, times and historical change” (Basu *et al.*, 1993, p. 74). Differences in life-chances within the categories “Hindu” and “Muslim,” however, are likely to be as significant as variations between the two categories. Essentializing Hindu and Muslim demographic behavior, though, is part of the Hindu Right's strategy to gain and retain political power, by creating an “Other” and reinforcing their own claim to represent all who regard themselves as Hindu. Given the Hindu Right's leading role in contemporary Indian politics, we would be evading our academic responsibilities if we refused to confront their myths of population out of control—even if, in doing so, we risk being mired in the numbers game (compare Basu, 1996, p. 154).

(a) *Islam and family planning*

An enduring feature of the Hindu Right's anti-Muslim rhetoric is that Indian Muslims have a grand plan to render Hindus a minority in their “own country,” a claim that relies heavily on asserting that Muslim fertility is

significantly higher than that of Hindus. Since the early 1970s, Hindu Right propaganda has also vigorously insisted that Indian Muslims are anti-national because of their supposed refusal to adopt “modern” contraception (for example, Hendre, 1971 and Prakash, 1979; see Basu, 1996, 1997; Wright, 1983 for appraisals). Indeed, some Hindu religious leaders, with support from communal political parties (Jana Sangh [now BJP] and Shiv Sena), have advocated population growth for Hindus to avert the purported threat of being outnumbered (Mandelbaum, 1974, p. 105).

The view that Islamic doctrine opposes family planning and that Muslims docilely respond to their religious leaders’ propaganda draws on (and feeds into) a much wider pattern of portraying Muslims as dominated by Islam. Yet comparable assumptions are rarely made about adherents of other religions. Extrapolations are drawn from a few Islamic theologians, with little thought to their social locations, to disputes among different schools of Islamic thought, to changes through time, or to the complex relationships between theology and everyday social practices.⁵

Through the centuries, Islamic texts have lent themselves to diverse theological stances on family planning. Omran (1992), for instance, focuses on the lack of clarity in Islamic texts over the acceptability of different forms of contraception, noting that some sources condone *coitus interruptus* and suggesting (by extension) that all forms of nonterminal family planning are acceptable. Some theologians have regarded abortion as acceptable before “ensoulment,” which occurs after the third month of pregnancy. Some authorities consider wanting to avoid poverty or to raise family living standards unacceptable motives for contraception, whereas wishing to avoid endangering the mother’s health or transmitting a serious disease can be morally justified. Theological objections have been raised about sterilization—that the permanent prevention of conception signifies a lack of faith in God’s capacity to provide—and it seems not to be actively endorsed by any school of Islamic jurisprudence (Khan, 1979, p. 184–191).

In South Asia, Maulana Maudoodi argued in *Radiance* (the Jama’at-i Islami journal) that birth control produces sexual anarchy (Wright, 1983).⁶ In Pakistan, though, Maudoodi’s position was not influential, and other Islamic leaders there have not denounced sterilization

in particular, despite their generalized opposition to the government’s family planning programs. In Bangladesh, the steep decline in the Total Fertility Rate and the rise in contraceptive prevalence rates since the early 1970s undermine the view that doctrinal opposition prevents South Asian Muslims from adopting family planning (Caldwell, Barkat-e-Khuda, Caldwell, Pieris, & Caldwell, 1999; Levin, Caldwell, & Barkat-e-Khuda, 1999). In several other Muslim-majority countries, large numbers of men and women have undergone sterilization, including Iran where it was banned after the Revolution of 1979 but made available again after 1989 (Obermeyer, 1994).

In India, some Muslim religious leaders have suggested that the political Emergency (1975–77) was an era when family planning staff and other government workers disproportionately targeted Muslims for sterilization. Some Muslim clerics, indeed, have claimed that family planning (especially sterilization) is contrary to Islam, but their failure to endorse the Indian government’s family planning program reflects the politics of minority status. Indian Muslims’ fertility behavior cannot be attributed to a supposedly universal and timeless Islamic condemnation of contraception in general, or of sterilization in particular (Basu, 1996, pp. 139–141). In any case, if Muslim religious leaders in India condemn contraception, we must ask how far their audiences take this into consideration in their own fertility behavior (compare Cassen, 1978, p. 56). This question is further complicated by the Indian family planning program’s obsession with sterilization (see below).

(b) *The “backward” Muslim woman and polygamy*

From the mid 19th century, Christian missionaries and local reformers alike condemned the seclusion of women (*purdah*) for causing health problems, endangering women in childbirth, preventing girls from attending school, and ensuring that their “backwardness” would continue to undermine their role as mothers (Lal, 1999; Minault, 1998; Savage, 1997). The most strictly secluded were women from the elite classes, whether Muslim or Hindu. In practice, then, *purdah* was not an Islamic institution, though it was often (erroneously) said to have been introduced into India by Muslim invaders.

A key strand in the Hindu Right's recent rhetorical demonization of Indian Muslims is how purportedly Islamic institutions, such as *purdah*, victimize Muslim women and perpetuate their "backwardness." This echoes (though with a communalist spin) contemporary demographers' preoccupations with the links between women's "autonomy," girls' schooling, and fertility (see below). Hindu Right claims about Muslim women's "backwardness," however, have generally been more indirectly linked to fertility through discourses on polygamy. Muslim men are stereotyped as more sexually active than Hindu men and as wanting more sexual partners and more children from them. Muslim women are cast as victims of Muslim men's (supposedly) excessive sexual appetites.⁷

Yet the proportion of Muslim men with more than one wife is small and about the same as or less than among Hindus (Krishnakumar, 1991; National Committee on the Status of women, 1975, pp. 21–23, 40–42). In rural Bijnor, polygamy is rare and it occurs about equally among Muslims and Hindus. Ethnographic accounts of Indian Muslims generally make only passing reference to polygamy. In practice, of course, there could be many polygamous marriages among Muslims only if Muslim men were marrying much younger women, or if (as the Hindu Right sometimes claims) women from other "communities" were converting and marrying Muslim men. There is, however, no credible evidence of conversions on a scale sufficient to create significant demographic shifts. In addition, the suggested link between polygamy and raised levels of fertility is problematic: what little evidence there is suggests that women in polygamous marriages usually have fewer not more children than comparable women in monogamous unions (Basu, 1996, pp. 138–139; 1997, pp. 10–11).⁸ In any case, polygamy has long been extremely contentious among Muslims. Conservatives generally say that Islam *permits* but does not actively encourage polygamy. Modernists argue that the Qur'anic verse that apparently permits polygamy sets such stringent conditions (that a man treat his wives equally) that it is effectively a prohibition, unless there are exceptional circumstances.⁹

Hindu Right rhetoric, then, rests on two crucial but false assumptions: that polygamy is common among Muslims but unknown among Hindus and that it is associated with high levels of fertility. Nevertheless, in 1968, the *Organiser* (the RSS journal) asserted that Muslims were

using polygamy to activate the population bomb as a war tactic against Hindus (Bacchetta, 1994, p. 198). During the Babri Masjid campaign of 1986–92, Sadhvi Saraswati claimed that polygamy "turns Muslim women into sexual objects and breeders," that for every five Hindu children born, Muslims have 50 and that Hindus would become a minority in India within 25 years (quoted in Basu, 1999, p. 173; see also Basu, 1996, p. 137), and Sadhvi Ritambara asserted that

The Muslims got their Pakistan. Even in a mutilated India, they have special rights. They have no use for family planning. They have their own religious schools. What do we have? An India with its arms cut off... The state tells us Hindus to have only two or three children. After a while they will say "do not have even one." But what about those who have six wives, 30–35 children, and breed like mosquitoes and flies? (quoted in Kakar, 1996, p. 225–226).

The Indian government's family planning slogan advocating a small family norm for everyone—"ham do, hamāre do" ("we two, our two") with its logo of a couple with their two children—has a public presence across India, from the walls of government buildings to postage stamps and railway tickets. The Hindu Right's distorted appropriation of this slogan neatly encapsulates their fallacies about Muslims and polygamy. A VHP pamphlet published in 1990 contained the riposte: while Hindus (supposedly) obey the government's slogan, Muslim men allegedly have four wives and say, "ham pāñch, hamāre pachīs" ("we five, our twenty-five"; see Gupta, 2002, p. 4; Sarkar, 1993, p. 165).

This slogan and the assumptions that Islam is set against contraception and that Muslim men marry polygamously have attained a taken-for-granted quality. Paola Bacchetta and Pradip Datta (personal communications) both suggest that such claims do not figure as centrally in official publications of Hindu Right organizations (such as the RSS) as in verbal discourse, whether interviews or speeches made at rallies and other gatherings. Moreover, what Datta (1999, p. 23) has termed "a trope of extinction" has become part of the "common truth, a product of social 'good sense'" (Datta, 1993, p. 1305) beyond Hindu Right activists (see also Wright, 1983). Certainly, we often heard views such as these in Bijnor in 1990–91 and again in our subsequent research in 2000–02, whether from urban middle class people, or from villagers such as Charan Singh. Few were paid-up

or active members of any Hindu Right organization (neither was Charan Singh).¹⁰ Yet comments were often larded with the “we five, our twenty-five” slogan, which was also blasted out by the tannoy system at the BJP offices in Bijnor town during the general election campaign in Spring 1991. Even some Muslims we talked to believed that Muslims would outstrip Hindus in the foreseeable future.¹¹ Crucially, the Hindu Right’s narrative rests on the belief that Indian Muslims’ fertility behavior is part of a plan to outnumber Hindus, a claim that rests on several demonstrably false premises and that seriously misrepresents complex social processes that influence fertility.

3. DEMOGRAPHERS ON FERTILITY DIFFERENTIALS

By contrast, demographers generally do not regard Islamic ideology as a key factor or, indeed, the categories “Hindu” and “Muslim” as sufficiently homogeneous for fertility differentials between them to be meaningfully analyzed.¹² Demographers systematically disaggregate the categories “Hindu” and “Muslim” along socioeconomic lines—region, place of residence (urban or rural), occupation, education (especially of the mother), class—rather than positing essential differences on the basis of religion *per se*. Thus they tend to undermine the very notion of “Muslim fertility.” Further, they examine the impact of polygamy, along with, for instance, age of marriage, length of breast-feeding, length of postpartum sexual abstinence, or other periods of sexual abstinence. But they treat these as proximate variables affecting fertility without making any assumptions about people’s intentions. In recent years, demographers have also focused on gender politics, but in a decidedly different fashion from the Hindu Right. The findings of academic demographers, then, provide compelling critiques of the Hindu Right’s simplistic claims about Hindu–Muslim differences in fertility behavior.

(a) *Region, class, and fertility*

At the macro-level, Indian demographic statistics indicate that Muslim fertility rates are somewhat higher than those among Hindus, and that Muslims make up a (slowly) growing share of the Indian population (up from 10.5% in 1951 to 12.5% in 1991). Apart from some

small-scale surveys and local case studies, demographers have mainly relied on special national surveys (Registrar General India, 1982; International Institute of Population Sciences, 1995) and the 1981 and 1991 national censuses for their analyses of inter-religious fertility differentials in India. Selecting only one indicator (total fertility rate, TFR), the Infant and Child Mortality Survey produced all-India estimates for 1978 for rural Muslim TFRs about 12% above those of Hindus (5.01 compared to 4.48) and somewhat more than 12% for urban residents (3.98 compared to 2.97).

Disaggregated figures, however, provide a more complex picture. The regional distribution of Muslims is uneven: in 1981, 36% of them lived in Bihar and Uttar Pradesh, which accounted for only 27% of India’s total population. These northern states have higher fertility and mortality rates than the rest of the country: the 1978 estimated rural TFR for Muslims was 6.39 and for Hindus was 5.82 in UP, and in Tamil Nadu the TFRs were 3.64 and 3.43 respectively; the urban rates were 3.88 and 3.21 in UP and 3.24 and 2.67 in Tamil Nadu. That is, Hindu fertility rates in north India are often higher than Muslim rates in central, eastern, and south India.¹³ The national surveys also show sizable statistical effects for the literacy level of the mother, age at marriage, and total household expenditure, all of which are greater than the inter-religious differentials.

Within north India, Muslims are more urbanized than Hindus (in UP and Bihar 29% of Muslims live in towns, whereas only 15% of Hindus do), but urban Muslims disproportionately occupy poorer housing areas with lower levels of public health infrastructure and have lower-paid jobs than their Hindu neighbors. More than half the urban Muslims are reported to have incomes below the poverty line, compared to 35% of urban Hindus. In 1978, 79% of rural Muslim households owned less than one hectare, compared to 68% of rural Hindu households (Krishnakumar, 1991; Shariff, 1995; Sridhar, 1991). In brief, Muslims are poorer and less educated than Hindus, as well as being distributed differently across the country (Registrar General India, 1982).

The independent effect of religion, however, seems not to have been analyzed using either of the large national surveys. Published analyses of the 1992–93 data provide all-India comparisons of TFR (4.41 for Muslims and 3.30 for Hindus) but urban and rural rates by state have

apparently not yet been systematically produced (Moulasha & Rao, 1999).¹⁴ Regional differences in fertility levels are still clear-cut (Mari Bhat & Zavier, 1999). Multivariate analysis of the independent role of religion has been done only for contraceptive use—and then only controlling for education and region separately, and not for socioeconomic status (Ramesh & Retherford, 1996). Large-scale survey data, then, indicate that there are inter-religious fertility differentials, but not whether they would remain or how large they would be after controlling for other important variables.¹⁵ The 1981 and 1991 census data, however, do present correlations between district-level characteristics and fertility, mortality, and female disadvantage (for example, Drèze & Murthi, 1999; Kishor, 1993; Mari Bhat, 1996; Mari Bhat & Zavier, 1999; Murthi, Guio, & Drèze, 1996). Sometimes religion is not mentioned at all (for example, Murthi *et al.*, 1996) or only in passing (Kishor, 1993). Mari Bhat (1996) and Drèze and Murthi (1999), though, find that the Muslim proportion of a district's population correlates positively with fertility indicators, even after controlling for other indicators.¹⁶ In these and other studies, urban-rural residence, women's labor force participation, and mother's schooling have been the key indicators analyzed. Though the details are complex, demographers' analyses of large-scale data sets controlling for a range of social and economic variables significantly (but not wholly) reduce the Hindu-Muslim fertility differentials much trumpeted by the Hindu Right.

(b) *Women's "autonomy" and fertility*

Since the early 1980s, "women's autonomy," or their capacity to make important decisions about their own lives, has been a preoccupation in demography. Most recent discussions focus on domestic politics—marriage arrangement, women's control over domestic resources, controls over women's mobility—as in Dyson and Moore's (1983) classic contribution. During the 1980s, labor force participation was considered crucial in enhancing women's domestic position and was central to discussions of regional differences in women's autonomy and discrimination against girls (Bardhan, 1974; Miller, 1981). But the unreliability of indicators of labor force participation derived from the census proved a major limitation. Recently, only Kishor (1993) has treated it seriously as an explanatory variable, although Moulasha and

Rao (1999, p. 3048), using the National Family Health Survey, note that "only 15% of the Muslim women participated in work (economic activity) whereas for Hindu women the figure was 34%."

One of the most robust statistical relationships, however, is between girls' schooling and fertility decline. During the 1990s, extending girls' access to schooling became central in policy initiatives aimed at enhancing young women's autonomy, particularly with respect to fertility decision making. For India as a whole, survey and census data alike show that Muslims have less schooling than Hindus, reflecting and contributing to Muslims' generally weaker economic positions. Age for age, in rural and urban areas alike, a smaller proportion of Muslim than Hindu children is currently attending schools of any kind (Krishnakumar, 1991; Shariff, 1995). Up to age nine, approximately equal percentages of Hindu and Muslim boys (urban and rural) attend school; for girls, the situation is comparable, although the percentages of girls at school are lower. For older rural children, however, the gap between Muslim and Hindu enrollment levels widens markedly, especially for girls. In part, this reflects the relative concentration of Muslims in north India, where overall school enrollment rates are lower than elsewhere. Within north India, however, Muslim schooling rates are below those of Hindus in the same class position, especially for schooling beyond primary level.

Some demographic literature on the Middle East claims that features of gender politics that supposedly reduce Muslim women's autonomy (particularly seclusion) are key to understanding continuing high levels of fertility in the region.¹⁷ Generally, though, analyses of women's autonomy have not regarded religious allegiance as a key issue. Krishnakumar (1991) and Basu (1996) are unusual in the emphasis they place on lower levels of literacy among Muslim women in India as a cause of higher fertility. Dyson and Moore (1983, p. 53), however, conclude that religious identification has relatively little influence on female autonomy, their main explanatory variable for demographic differences between north and south India.

Population increase, of course, is also a matter of mortality and migration. There is little information on mortality rates by religion in India. Infant death rates are higher in the north than in the south, yet, for the country as

a whole, Muslim infant mortality rates were about 109 per 1,000 live births in 1978, compared with about 121 per 1,000 live births for Hindus, a difference of 10–12% (Registrar General of India, 1981). Moulasha and Rao (1999, p. 3049) quote figures suggesting a child survival rate of children born to Muslim mothers 20% above that of children born to Hindu mothers. Unfortunately, there are few good multivariate analyses that combine factors like region, rural–urban residence, schooling of parents, and economic position to determine the net relationships between religion and mortality. Considering the large contribution of infant and child mortality to total mortality, though, Muslim total mortality might also be around 10% below that of Hindus. If so, this could be almost as significant (in terms of differentials in population totals) as fertility differences but, unlike fertility, mortality has not been politicized by the Hindu Right. For most of India, migration rates are too small to be demographically significant (Moulasha & Rao, 1999, p. 3047), but migration has become politically contentious, with widespread anxiety about the infiltration of people from Bangladesh. In western UP, though, local migration patterns have not been an issue in communal politics.

In sum, demographic analyses indicate that Muslim and Hindu fertility levels alike are higher in rural areas, in north India, and among the poor and the poorly educated, than in urban areas, in south India, or among wealthier and educated couples, especially when the wife has more than eight years of schooling. After taking account of differences in the proportions who marry and in the age distributions of the two categories, demographic differences between religious communities can be largely (though not wholly) understood with reference to social and economic factors. Moreover, all these fertility rates are well below “natural” fertility and fertility was declining for all groups at about the same rate in the 1980s and 1990s, albeit from different starting points. Even if Muslim fertility rates fall more slowly and Muslim mortality rates remain lower than Hindu rates, the Muslim proportion of India’s population is unlikely to reach 15% by 2021.¹⁸ This contrasts markedly with Hindu Right rhetoric that reifies the categories “Hindu” and “Muslim,” assumes a homogeneity and potency to Islamic ideology, and claims that Muslims will soon outnumber Hindus.

4. MUSLIM AND HINDU IN RURAL BIJNOR

We largely concur with demographers who argue that fertility differentials in India can mainly be accounted for by analyzing social and economic variables and that the role of religion *per se* is relatively limited. Yet something still needs to be explained, for fertility differentials remain—albeit much smaller than those implied by Hindu Right rhetoric—even after regional and socioeconomic variables have been taken into account.

In analyzing our Bijnor data, however, we have become increasingly dissatisfied with conventional macro-demographic analysis. First, most demographers rely on special surveys or district-level data from the census, but problems arise from working with units of analysis as large as a district (with perhaps several million people). When there is diversity below this level, excessive aggregation of data may mask variations between and within localities. Explanations of differences in fertility behavior, then, cannot be meaningfully carried out at the level of nation or even region. Second, demographers conduct elaborate statistical techniques to try to separate out the main elements in a causal model of demographic change, as in the recent emphasis on the widespread, statistically significant correlation between girls’ education and low fertility. But our fieldwork suggests that this correlation is not necessarily straightforwardly causal (see below).¹⁹ In general, detailed micro-studies using local-level data and dealing with meaningful social groupings provide a better window into the fine-grain and locally specific processes that are likely to elude large-scale surveys. Differences exposed by large-scale statistical analyses, then, may suggest possible hypotheses, but not final answers.

Some social demographers, indeed, advocate amplifying data on measurable features of individuals (age at marriage, desired family size, and so on) derived from large-scale surveys with small-scale studies of the social relationships that link individuals’ decisions (or non-decisions) to the collectivities to which they belong. Our experiences in rural Bijnor endorse Greenhalgh’s view that we must “situate fertility, that is, to show how it makes sense given the socio-cultural and political economic context in which it is embedded” (Greenhalgh, 1995, p. 17). By locating people’s approaches to fertility in the context of rural Bijnor, our

“situated” account challenges both Hindu Right rhetoric and conventional demographers’ accounts. We focus here on local perceptions of the family planning program, examine aspects of gender politics and women’s autonomy at the domestic level, and, finally, outline local understandings of social and economic exclusion.

We conducted village surveys in 1990–91 and interviewed people from many different caste and class backgrounds in rural Bijnor district. The material here, though, mainly concerns two roughly comparable caste groups, Sheikhs (Muslim) and Jats (Caste Hindu).²⁰ Muslims in Bijnor district, especially in urban areas, are particularly associated with artisanal and laboring activity. Rural Sheikhs, however, are the prime Muslim landowners in this part of the district, as are the Jats among caste Hindus. Among Sheikhs and Jats alike there are class differences (assessed largely in terms of land ownership). Even when class is held constant, though, Sheikh fertility is higher than Jat fertility. Hence, we must ask if there are features of the local situation that affect the Sheikhs as *Muslims* and contribute to their higher fertility.

(a) *Muslims and family planning in rural Bijnor*

The image of Muslims rejecting family planning because of Islamic doctrine and an ambition to outnumber Hindus is central to the Hindu Right perspective. Our data suggest that Sheikhs (indeed Muslims in general in rural Bijnor) did resist the government family planning program, but mainly for reasons of quality of service and the contraceptive technologies being offered.

We frequently observed the supercilious and disdainful manner of medical and paramedical staff (who were usually Hindus from relatively high caste urban backgrounds), especially with respect to Muslim and poor Hindu villagers. Semipublic consultations compromised patient privacy and confidentiality, and women were often chided for their childcare practices or their repeated pregnancies. Local sensibilities about family planning were particularly acute, because government health staff have had “targets” (the number of “cases” they should motivate for family planning) throughout much of the period since the mid 1970s. Consultations would routinely be diverted onto family planning (for example, Najma’s visit to an ophthalmologist: Jeffery & Jeffery, 1996a, pp. 53–68). Many Muslims to whom we talked

thought they were singled out by family planning workers, as they believed they had been during the 1975–77 Emergency, when health workers and other government staff were put under greatest pressure to meet family planning targets. Health staff and many Muslims had diametrically opposed understandings: the former often commented adversely on Muslim views about family planning, the latter described the family planning program as a government initiative to eradicate Muslims. At the same time, health services in Muslim-dominated villages were of poorer quality than in Hindu-dominated villages. Many Muslims complained that they were ignored in public health campaigns, such as immunizations, although some regarded such services as mere sweeteners to persuade people to accept family planning.

Mistrust of government health and family planning services was undoubtedly exacerbated by a bias toward sterilization (overwhelmingly female sterilization since the late 1970s), despite supposedly offering a “cafeteria” service where people would select the family planning method that best suited their needs. Family planning workers not only had sterilization targets but received incentive payments for every case they motivated (with lower payments for IUCD or intrauterine contraceptive devices); family planning acceptors also received small sums after sterilization or IUCD insertion. Pressure on staff was such that they sometimes exaggerated their achievements in the health center records (see Jeffery, Jeffery & Lyon, 1989; Narayana & Kantner, 1992 provide the national picture). Until the late 1980s, people in rural Bijnor generally thought that family planning and sterilization were synonymous and that family planning staff could or would offer only sterilization. Certainly, some women had IUCDs inserted and the contraceptive pill was available from pharmacies in Bijnor town, but a few women who wished their IUCD to be replaced or who wanted prenatal tetanus injections reported being pressured to become a sterilization “case.” Many people, especially poor Hindus and Muslims, feared being coerced into adopting terminal methods such as sterilization. Although most women would have preferred spacing methods, mainly because of their fear of child mortality, it was widely considered futile to try to obtain them from family planning staff.

This led to popular resistance, as witness the requests we received during our research for

alternative contraceptive methods, from Muslim and Hindu women alike. Certainly, rather than wanting numerous children, many of the Muslim women we talked to wished to limit their fertility, often pointing to the health costs to themselves and their children of repeated childbearing. Crucially, too, our data indicate significantly *higher* rates of infant and child mortality among rural Muslims in Bijnor than among comparable Hindus, in contrast to all-India statistics.

For Muslims in rural Bijnor, part of the story, but only a part, was certainly their belief that Islam prohibits sterilization and that they should not use the family planning method most readily available through the government health services (Jeffery & Jeffery, 1996b, 1997). But our field notes indicate that their reactions to the family planning program cannot be largely (leave aside wholly) attributed to their understandings of Islamic doctrine. The Muslims we talked to in rural Bijnor approached fertility limitation in a similar fashion to that of most of the other groups in the locality, with no generalized resistance to *spacing* methods of fertility limitation, but with an aversion to terminal methods and a mistrust of the government's family planning program. In other words, our research indicates that many women's needs for acceptable contraception were far from being met.

The Jats presented a striking contrast to this picture. Far more of them were actively limiting their families than were Muslims as a whole (including Sheikhs) and Hindus from the smaller, poorer, and lower castes. Of the Jat women we interviewed in 1991, 59% of those aged 35–45 and 42% of those aged 25–35 were using some form of modern contraception. They accessed health care and contraceptive services, including pills, IUCDs, and female sterilization via government family planning services as well as local private doctors. Does this mean, then, that instead of trying to account for the fertility differentials between Sheikhs and Jats in rural Bijnor in terms of Islamic doctrine, we should focus on women's capacity to implement their family planning preferences?

(b) *Women's autonomy, education, and fertility in rural Bijnor*

Deconstructing the household can show that household members (such as husbands and

wives) may have conflicting as well as common interests.²¹ Gender politics at the household level, then, are likely to be crucial in many matters, including family planning decisions: as Greenhalgh (1995, p. 14) argues, reproduction is a "deeply gendered process." In recent demographic discussions of women's autonomy and fertility decline, girls' education has been a central concern.

In line with the national picture outlined above, girls from different castes and communities in rural Bijnor tend to have had different schooling experiences. Most Jat women had attended school. Sizable numbers, disproportionately from the wealthiest households, had obtained post-school qualifications such as BA or MA. By contrast, not even Sheikh women from rich peasant households were likely to have been educated outside the home. Those few who did attend school usually went to a *madrasā* (mosque school) rather than a secular school. Might the fertility differentials between rural Sheikhs and Jats that remained after controlling for class be attributed to different schooling experiences and different levels of autonomy among adult women?

Unfortunately, demographers have tended to treat both "education" (usually glossed as "years of schooling") and "autonomy" as black boxes that are simple variables consistently and causally linked to one another and to fertility. Thus Drèze and Murthi (1999, p. 3), for example, find that "women's education emerges as the most important factor explaining fertility differences across the country and over time." A local-level perspective, however, indicates that "autonomy" and "education" and their inter-relationships are far more complex than this.

In rural Bijnor, it is doubtful that schooling experiences in themselves provided women with the capacity to act autonomously. School curricula, styles of classroom interaction, and the body language inculcated in girls at school seemed more likely to sustain conventional gender hierarchies than to enhance girls' autonomy, in the sense of capacity to think and act independently. At a *madrasā*, girls had to cover their heads and behave demurely while learning to recite the *Qur'an Sharif*; the wider curriculum did not always include Hindi and other subjects that might give access to knowledge beyond *Islāmiyāt* (Islamic ideas and history); and *madrasā* schooling was terminated once girls reached puberty. Secular schools also provided little scope for ensuring that pupils graduated with enhanced autonomy. In

addition, many rural women who studied to BA and MA levels had done so as "private" rather than "regular" students, that is by correspondence courses at home—hardly an effective way of honing the skills and confidence to deal with the world beyond the home that are so often considered an important consequence of school attendance. Moreover, it was abundantly clear that parents did not send daughters to school in order to create autonomous women. Obtaining qualifications for paid employment was almost never considered (although some people said qualifications might enable a woman to stand on her "own legs" after a calamity such as widowhood or divorce). Rather, parents saw their daughters' schooling as a newly important (and quite costly) asset in the marriage market.

Measuring "education" in terms of "years of schooling," then, simply cannot grasp the local meanings attached to girls' schooling or the contrasts between different types of schools or different locations in which girls study. In any case, even if schooling could enable girls to *think* more independently, educated Jat women had no more control over the choice of their marriage partner or the timing of their marriage than women with little or no schooling. In rural Bijnor, young married women of all castes and communities, for whom decisions about fertility were particularly salient, operated within broadly comparable forms of domestic gender politics.

It is generally assumed that *purdah* disempowers women, for it restricts their mobility outside the home and may imply their lack of involvement in paid employment that could provide independent income. For the Hindu Right, *purdah* is a marker of Muslim women's "backwardness," but many ethnographic studies report on seclusion and veiling practices among Hindus, albeit often pointing to subtle differences between Hindu and Muslim *purdah* (see Jacobson & Wadley, 1995; Mandelbaum, 1986, 1988; Papanek, 1982). Certainly, our data do not support an argument that Sheikh women (or indeed Muslim women as a whole) were more restricted by seclusion practices than Jat women (or Hindu women in general). Rather, seclusion practices reflected local ideas about family honor that were common to Hindus and Muslims.

Adult women's mobility beyond the domestic domain reflected differing work demands (relating primarily to the household's class

position) and household composition and a woman's position in it (for example, as sole adult woman, or as mother-in-law or daughter-in-law), not religious allegiance. In the wealthiest rural households, women worked inside the house but rarely outside. In poorer households, women often had to work outside, whether as family labor or as employees of wealthier households in the locality, but such mobility was limited to the task at hand and did not imply enhanced autonomy or freedoms. One feature of gender politics remained constant, however: land, the major economic resource, was owned (albeit very inequitably) by men and not by women. Few women had any independent income; employed women worked out of necessity but usually earned very little as they had no marketable skills aside from domestic or farming work. Sheikh and Jat women as a whole, then, were not systematically differentiated in terms of mobility beyond the home, access to land and independent income: such contrasts as were present reflected class rather than community differences.

Our data highlight an additional problem with assuming that "education" straightforwardly enhances young married women's autonomy in their marital homes. The most educated Jat women were married into the richest households and were expected to be home-based. They were also more likely to be married to an only son and thus required to share a household with their mother-in-law. Yet women of all castes and communities thought sharing a household with the mother-in-law was more constricting than running their own household. Thus, girls' education neither correlates convincingly with differences in women's "autonomy" nor provides us with much insight into the fertility differentials between Jats and Sheikhs in rural Bijnor.

Further, low state school attendance by Sheikh and other Muslim girls cannot readily be explained by *purdah* restrictions (or Muslim "backwardness"). For many Muslim families, poverty was an important restraint on sending children to school. For more wealthy Muslims, though, low state school attendance by girls reflected (among other things) the tendency for state schools not to be located in Muslim villages or Muslim areas of mixed villages. In addition, Muslim parents believed that Muslim girls were sexually and communally harassed at school and that the *madrasa* provided a more appropriate curriculum in a more protected environment.

None of these observations means that gender politics within the household are unimportant in decisions about girls' education (or fertility) in rural Bijnor. They do require, though, that we locate individual behavior and household dynamics within social and economic processes and relations of difference and inequality *beyond* the household (McNicol, 1994).

(c) *Perceptions of exclusion in rural Bijnor*

For Greenhalgh (1995, p. 21), fertility transitions are the "products of changes in class-specific opportunity structures in response to transformations of global and regional political economies." Here, then, we want to account for the fertility differentials between Sheikhs and Jats in rural Bijnor in terms of their contrasting locations in local social and economic structures. We focus on the social and political implications of labeling oneself and being labeled by others as "Muslim" in rural Bijnor, not on Muslims as adherents of stereotyped Islamic spiritual beliefs and doctrinal commands.

Dyson (1991) considers that high fertility among poor rural populations does not need much explanation, largely because few people are making active decisions about family sizes. Fertility levels may be lower than "natural" fertility, but they may reflect unintentional factors such as age at marriage, length of breast-feeding, or sexual abstinence, rather than purposeful contraceptive use. By contrast, low or declining fertility in some sectors of the population does need explanation.

Were some Hindus in rural Bijnor, notably the Jats, experiencing social processes that led them to favor small families, whereas Muslims were not? As elsewhere in India, Muslims in rural Bijnor tended to be poorer and less educated. But school attendance by Muslim boys was lower than for Hindu boys even when class was held constant. Sheikh and Jat parents alike saw boys' schooling primarily as a resource in the job market and secondarily as an element in marriage arrangements. Charan Singh's comment quoted above is typical: "Unless they get a good job, what benefit will there be from the education?" Jats were generally optimistic that schooling enhances (though does not guarantee) the chances of obtaining white collar employment. Crucially, however, Sheikhs in Bijnor did not believe that their sons would get good jobs, no matter how much schooling they

received. Sheikhs generally did not spend as much money on their children's schooling or put as much pressure on their children to attend school as did the Jats. Although Sheikh parents' views about schooling might adversely affect their sons' job prospects, several pointers suggest that the reverse is more plausible—that Sheikhs' experiences of exclusion from the job market led them to put a lower value on schooling than comparable Hindus.

First, Muslims are not covered by job quotas in government services, in contrast to the Scheduled Castes and Tribes and the Other Backward Castes. Nowadays, the practical significance of such exclusion is questionable, since there are very few "reserved" posts available. Nevertheless, many Muslims in Bijnor felt that their exclusion reflected other aspects of the Indian government's attitude toward Muslims. Second, our Muslim and Hindu informants generally claimed that job applicants in the public and private sectors alike needed either influence (*safārish*) through caste, kinship, or religious community networks or cash for a bribe (*rishwat*) or both to have much chance of being appointed. Firm evidence for these claims is hard to find. Yet the *belief* in such processes was crucial in parents' calculations about the value of investing in their sons' schooling. The absence (perceived or real) of Muslims from key positions damaged Muslims' access to future appointments, directly (because Muslim applicants have few people over whom they can exert "influence") and indirectly (by disheartening students and their parents at crucial stages of their progress through schooling). This also applied, of course, to poor Hindus who lacked the necessary networks and cash.

In addition, schooling is not standardized. Wealthier families, rural as well as urban, would pay for private schooling and for "tuition" after school hours for their sons, and expect women of the family to supervise children's homework. Poor rural families, however, could access only poor quality schooling and their low levels of social and cultural capital also restricted what their sons could gain from schooling. Muslims were differentially disadvantaged by poverty, yet, unlike children from the Scheduled Castes and Tribes, they were not entitled to scholarships and fee waivers. *Madrasā* schooling may have been more financially accessible, but it fed only into theological training and employment as an *imām* for a few

able boys. Further, many Muslims believed that the ambience of mainstream schools was hostile to Islam and that Muslim children were subjected to harassment from other pupils and teachers alike. The uphill struggle to school their sons often hardly felt worthwhile, especially since the rewards, in terms of secure employment, seemed so elusive.

Girls' schooling certainly enters people's calculations about fertility and schooling, but not in the direct way posited by the demographic orthodoxy. Girls' schooling was valued for its contribution to marriageability, not employability. All girls' schooling was conditioned by the need to educate them to a level just below that of the kind of boy to whom their parents hoped to arrange their marriages. For many Jat girls, this could entail lengthy schooling and even college education. By contrast, Muslim girls' problems of harassment and of access to schools were compounded by the low levels of school attendance by Muslim boys, even among the relatively wealthy.

The crucial factor differentiating the fertility regimes of the Jats and the Sheikhs, however, was the Jats' relative success in the off-farm employment market. Most Jat men were limiting their families in pursuit of household goals: to educate their sons and place them in good employment, and to educate their daughters and arrange their marriages to educated young men with off-farm employment who would command increasingly larger dowries. Jat parents, then, obeyed the dictates of their pockets rather than the government's family planning slogan. As Charan Singh put it, "But children are too expensive: the everyday costs are so high I could not afford any more." By contrast, the Sheikhs lacked access to influential people and processes of exclusion from sought-after employment effectively reduced the costs of child-rearing (school fees, bribes, dowry, for instance). Sheikh men were not unthinking followers of *molwis* who (allegedly) told them to have large families. Rather, they remained locked into the agrarian economy. Investing in schooling made little sense and they had little incentive to limit their fertility—calculations that also applied to smaller and poorer Hindu and Muslim groups in rural Bijnor. In these respects, then, there was nothing essentially "Muslim" about the Sheikhs' fertility regime.

Many Sheikh and other Muslim women articulated views on fertility akin to those of their husbands, emphasizing Muslims' difficult

social and economic position. In addition, because of low school attendance, Muslim girls were available for marriage at younger ages than comparable Hindu girls. Many Muslim women were dismayed by the damage to their own health and the heavy workloads caused by lengthy childbearing careers. Muslim women wanting contraception often had no option but subterfuge. By contrast, Jat couples generally made family planning decisions jointly. Jat women's wishes usually coincided with their husbands' plans, although a few Jat women could not have as many children as they themselves wanted because their in-laws wished to prevent land fragmentation and limit the costs of child-rearing. In brief, Sheikh men did not especially oppress their womenfolk nor did Jat women have greater "autonomy." Rather, men's fertility rationales tended to prevail among Jats and Sheikhs alike.

5. CONCLUDING THOUGHTS

The communalization of politics in contemporary India has been buttressed by images of vulnerable Hindus (supposedly) doomed to be outnumbered. Assertions about Muslim fertility and about Muslim men's right to marry polygamously have been key to the Hindu Right's propaganda. Such communalist population myths are "short on facts, short on a proper understanding of the demographic situation—and short on honesty" (Krishnakumar, 1991, p. 94) and dangerous propaganda in the victimization of minority groups that contributes to creating the very differences that are used to justify demonizing Muslims.

During our first fieldwork in Bijnor in 1982–83, local Muslim commentaries on the family planning program and on boys' employment prospects already exposed Muslims' sense of marginalization and vulnerability. Political developments during the 1980s confirmed for them the government's ill intent toward Muslims and reinforced their distrust of the government machine. Muslims' insecurity was also exacerbated by sloganeering from the Hindu Right, by the communal disturbances in many parts of India during late 1990 (including in Bijnor: see Basu, 1995; Jeffery & Jeffery, 1994a,b) and after the Babri Masjid was demolished in 1992, and by the BJP's subsequent electoral successes.

These physical and symbolic relations of dominance and subordination generated a sense of insecurity that structured the lives of Sheikhs in rural Bijnor *qua* Muslims. The expense of schooling children and accessing employment was linked to fertility behavior just as much among Sheikhs as among Jat farmers, though with different outcomes. Far from higher fertility being an Islamic strategy for power or a plan to outbreed Hindus, it grew out of the Sheikhs' lack of power when compared with the locally dominant Jats.

Macro-level data, on which demographers typically rely, mask the fine-grain and complex processes that result in fertility differentials within a locality. The move away from the construction of meta-narratives of fertility be-

havior and toward "situated" accounts, however, offers the promise of greater explanatory potential that could be used to good effect throughout South Asia (though with the details certainly differing from our account here). The move would be important for this reason alone. Given the politicization of population issues in contemporary India, however, it has additional significance. "Situated" accounts allow us to undermine the Hindu Right's views on Indian demography. Their views have achieved widespread acceptance and lie behind many acts of everyday routinized violence and discrimination against Muslims in India. It is thus imperative and politically urgent to challenge the Hindu Right's myths of Indian Muslims' population out of control.

NOTES

1. Charan Singh is a pseudonym. This is our translation from the Hindi.

2. The Hindu Right is also known as the Sangh Parishad or "Family of the Sangh" (that is the Rashtriya Swayamsevak Sangh). Space precludes detailed analysis of differences in emphasis among these organizations.

3. On the Ayodhya affair and the communalization of politics in contemporary India, see Basu and Kohli (1998), Basu, Datta, Sarkar, Sarkar, and Sen (1993), Chakravarti *et al.* (1992), Engineer (1991), Gopal (1991), Hasan (1997), Jaffrelot (1996), Ludden (1996), van der Veer (1994), Vanaik (1997). The Ayodhya affair was linked to the Shah Bano case, which revolved around the rights of divorced Muslim women. Space precludes detailed discussion of this case. It raised many issues, including (for Muslims) state intrusion into the "private" sphere of religious minorities (that is echoed in Muslims' responses to the family planning program) and (for the Hindu Right) the trope of the Muslim woman as victim (that also comes up in relation to polygamy; see below). The following provide useful accounts of the case: Chhachhi (1991), Engineer (1987), Hasan (1989, 1993, 1994, 1999), Kishwar (1986), Kumar (1993), Mody (1987), Palriwala and Agnihotri (1996), Pathak and Rajan (1989). See also Jeffery (2001).

4. These allegations echo both the "Islamophobia" currently widespread in the West and classically Orientalist arguments (Obermeyer, 1992; Said, 1978). Sikhs and Christians have sometimes been accused of under-

mining Hinduism through religious conversions and faster population growth rates, but Muslims have been the target of most such claims in the 1990s (Krishnakumar, 1991; van der Veer, 1994; Wright, 1983).

5. Writing about the Middle East, Fargues (1993) provides a recent example of the links drawn between Islamic fundamentalism, dreams of world conquest, and high fertility rates. By contrast, Obermeyer (1994, p. 60) highlights the tensions in Islamic doctrines between egalitarian and inegalitarian views and the "tremendous complexity and diversity that is found in the Muslim world"; see also Ahmed (1982).

6. Similarly, reversing Hindu communalist messages, G.M. Shah argued that every Kashmiri Muslim should have a dozen children because they were dying out (Pai Panandikar & Umashankar, 1994). Shah's demography was as incompetent as that of his Hindu Right opponents: the Muslim proportion in Jammu and Kashmir was 57% in 1947 and 64% in 1981.

7. Widow remarriage was also central to early 20th century Hindu nationalist rhetoric. Muslim widows could remarry whereas Hindu widows prevented from remarrying within their own community were allegedly lured into marriages with Muslim men. Either way, it was claimed, Muslim numbers would grow at the expense of Hindus. With declining levels of widowhood and rising levels of widow remarriage among Hindus, however, widow remarriage is no longer as contentious as it once was (Gupta, 2002, pp. 318, 324; see also

Datta, 1993, Davis, 1951, pp. 79–82, Mandelbaum, 1974, p. 35).

8. A first wife's failure to bear children (particularly a son) is a common reason for polygamy. Since male infertility may be the problem, second or later wives may also have difficulty in having children. Further, men marrying polygamously are generally older than those marrying for the first time and women in polygamous marriages are more likely to be widowed before menopause.

9. See Ahmad (1967, pp. 63, 73, 95) for a discussion of 19th century modernist views and Engineer (1992, pp. 22, 98ff, 154ff) for a contemporary argument against polygamy.

10. Several young Jat men from his village, however, became involved in the Ayodhya campaign in late 1990, courting arrest in Bijnor town and attending political rallies.

11. Such claims make no sense nationally but local situations may seem less clear-cut. In Bijnor, for instance, "caste Hindus" have gone from 46% of the total district population in 1921 to about 37% in 1991, and the Muslim share has risen from 36% in 1921 to 40% in 1991, despite some migration of Muslims to Pakistan in the late 1940s.

12. Goldscheider (1971) distinguishes three hypotheses linking fertility and religion. The first, "particularized theology," attributes the effects of a religion to its doctrines about birth control and family size. The second, "characteristics," explains religious differences in fertility through the social, economic, and demographic characteristics that the members of a particular religion happen to have. The third, "minority group status," explains lower fertility among minority religious groups as a result of their insecurities. A fourth model, introduced by Chamie (1981) describes "interactions" that may change the relationships between fertility and religion through time. As Knodel, Gray, Sriwatchin, and Peracca (1999, p. 163) point out, none of these is very satisfactory, either separately or together. We agree with their preference for a truly interactive model, one that acknowledges that "cultural, political, socio-economic, and historical factors interact with the relationship [between religion and fertility]" and that the "national and local context conditions the extent and perhaps nature of Islam's influence."

13. In these calculations, "Hindu" includes "Scheduled Castes" and "Scheduled Tribes."

14. For a critique of their general approach, see Jeffery and Jeffery (2000).

15. In a small-scale study of rural Koil tahsil (Aligarh District, UP) the fertility differentials of caste Hindus, Scheduled Castes, and Muslims could be almost entirely understood in terms of the impacts of income, education, child mortality, and age at marriage (Khan, 1991, pp. 110–115).

16. It is likely that Hindus in these Districts have higher fertility than Hindus in other Districts, and that Muslim fertility in these Districts is at least as high, and probably higher, but it is not clear why this might be.

17. Until the 1980s, Islamic countries in the Middle East were poor health achievers (relative to their wealth) and also, despite relatively high levels of female literacy, continued to have high levels of fertility. Caldwell (1986, p. 175) considered that "the central aspect of the relationship between Islam and mortality levels is undoubtedly the separate and distinctive position of women, operating partly through their access to education but also in many other ways." He has now shifted position somewhat, because fertility levels in the region have dropped dramatically without apparently being accompanied by major social and economic changes, especially in relation to women's autonomy (Caldwell *et al.*, 1999).

18. Drèze and Murthi (1999) conclude that Muslims and Hindus differ neither in their preferences for sons nor in differentials in mortality rates by gender (see also Murthi *et al.*, 1996). Sex ratios for Muslims in north India have historically been less "masculine" than those for Hindus, probably because of differences in mortality rates overall and the relationship of regional and socioeconomic factors to fertility (Agnihotri, 1997).

19. See Jeffery and Basu (1996) and Jeffery and Jeffery (1998) for critiques of this new orthodoxy and Drèze and Murthi (1999) for a response. We can note that neither Drèze and Murthi nor Mari Bhat (1996) elaborate on the correlation between high fertility and a high proportion of Muslims in a district, yet it is as strong as the links between girls' schooling and low fertility that are generally presumed to be causal.

20. This section draws on several of our publications on Bijnor, especially Jeffery (2000, 2001), Jeffery and Jeffery (1993, 1994a,b, 1996, 1996b, 1997, 1999), Jeffery, Jeffery, and Lyon (1989). As we indicated above, polygamy is extremely rare in Bijnor, so we do not discuss it further here.

21. See Agarwal (1997), Kandiyoti (1988, 1998), Sen (1990) for elaborations of these issues and also Kabeer (1999) on the problems of measuring women's empowerment and autonomy.

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